

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Plain Language Title: A Study to Learn About the Study Medicine Called PF-08046054/SGN-PDL1V Versus Docetaxel in Adult Participants With Previously-treated Programmed Cell Death Ligand 1 (PD-L1) Positive Non-Small-Cell Lung Cancer (NSCLC) **Official Title:** A RANDOMIZED, PHASE 3, OPEN-LABEL STUDY TO EVALUATE PF-08046054/SGN-PDL1V VERSUS DOCETAXEL IN ADULT PARTICIPANTS WITH PREVIOUSLY-TREATED PROGRAMMED CELL DEATH LIGAND 1 (PD-L1) POSITIVE NON-SMALL-CELL LUNG CANCER (NSCLC) **Short Title/Acronym:** PADL1NK-005 **Protocol Number:** C5851005 **NCT Number:** NCT07144280 **Posting ID:** 786873580939270071 **Trial Status:** RECRUITING (Currently recruiting participants) **Sponsor/Company Name:** Pfizer **Investigational Product:** PF-08046054 / SGN-PDL1V (anti-PD-L1 antibody-drug conjugate with MMAE payload) **Active Comparator:** Docetaxel (Trade Names: Docefrez, Taxotere; ATC Code: L01CD02) **Phase of Development:** Phase 3 **Medical Monitor:** [Name and Contact Information]

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To evaluate the Overall Survival (OS) and Progression-Free Survival (PFS) assessed by Blinded Independent Central Review (BICR) comparing the experimental arm (PF-08046054) and control arm (docetaxel) in participants with PD-L1 positive NSCLC. * **Secondary Objectives:** To evaluate Objective Response Rate (ORR) as assessed by BICR; Progression-Free Survival (PFS) as assessed by Investigator; Objective Response Rate (ORR) as assessed by Investigator; and to assess changes in Quality of Life (QoL), functioning, and lung cancer symptom response.

2.2 Background & Rationale The purpose of this study is to understand if PF-08046054 alone works well compared to standard-of-care docetaxel alone in participants with non-small cell lung cancer (NSCLC) with PD-L1 expression $\geq 1\%$. Patients must have had cancer progression during or after previous treatment with PD-L1 or PD-1 inhibitors, platinum-based chemotherapy, and targeted treatment regimen(s) for actionable genomic alterations (AGAs). Participants must have cancer that has spread through their body or cannot be removed with surgery or treated with definitive radiation.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Active Comparator (Docetaxel monotherapy) * **Blinding:** Open-label * **Randomization:** Randomized (like a flip of a coin to assign treatment groups)

3.2 Planned Enrollment & Duration * **Target N:** 680 evaluable participants * **Number of Sites:** Global multicenter (including US, Europe, Canada, Japan, RoW) * **Estimated Study Start:** 29-Sep-2025 * **Estimated Primary Completion:** 10-Mar-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically or cytologically confirmed diagnosis of locally advanced, unresectable Stage IIIB and IIIC not eligible for definitive chemoradiotherapy or metastatic (Stage IV: M1a, M1b, or M1c) NSCLC per the American Joint Committee on Cancer (AJCC) Staging Manual, Version 8.0, and the Union for International Cancer Control (UICC) Staging System. *Note: Participants with a neuroendocrine component or histology are not eligible.* 2. PD-L1 expression on $\geq 1\%$ of tumor cells based on local immunohistochemistry (IHC) testing with an assay utilizing the anti-PD-L1 monoclonal antibody clones 22C3 or SP263. 3. Participants who have NSCLC with known AGAs are permitted (e.g., estimated glomerular filtration rate (EGFR) mutations, anaplastic lymphoma kinase (ALK) translocations). 4. Able to provide tumor tissues for biomarker analysis: Archival specimen (preferably collected within 12 months after the last anticancer therapy) OR fresh tissue from a tumor lesion, if medically feasible. 5. Must have received the following therapies and progressed during or relapsed after receiving their most recent prior therapy: * *Participants with no known AGAs:* Must have received a platinum-based combination therapy for metastatic/recurrent disease and a PD-L1/PD-1 monoclonal antibody; OR experienced disease progression within 6 months of the last dose of platinum-based chemotherapy in the adjuvant, neoadjuvant, or chemoradiotherapy setting and received a PD-L1/PD-1 monoclonal antibody at any time. * *Participants with known AGAs:* Must have received at least 1 approved AGA-targeted therapy (and additional AGA therapy is not in the patient's best interest); received platinum-based combination therapy for metastatic/recurrent disease or progressed within 6 months of last dose in the adjuvant/neoadjuvant/chemoradiotherapy setting; and may have received PD-1 or PD-L1 monoclonal antibody.

4.2 Exclusion Criteria 1. History of another malignancy within 3 years before the first dose of PF-08046054, or any evidence of residual disease from a previously diagnosed malignancy. *Exceptions:* Malignancies with a negligible risk of metastasis or death (e.g., 5-year OS $\geq 90\%$), such as adequately treated carcinoma in situ of the cervix, non-melanoma skin carcinoma,

localized prostate cancer, ductal carcinoma in situ, or Stage I uterine cancer. 2. Active central nervous system (CNS) lesions are excluded (defined as untreated or symptomatic requiring corticosteroids >10 mg/day of prednisone equivalent within the previous 14 days). Participants with clinically inactive, definitively treated brain metastases (surgery and/or radiotherapy) are eligible if: on a stable dose of ≤ 10 mg/day of prednisone equivalent for at least >14 days; no evidence of clinical/radiographic disease progression in the CNS for ≥ 28 days after definitive therapy; and any higher-dose corticosteroid use occurred ≥ 28 days prior to Screening. 3. History of leptomeningeal metastasis. 4. Prior treatment with an anti-PD-L1 agent (where indicated per protocol) within 5 half-lives. 5. Previous receipt of an Monomethyl auristatin E (MMAE)-containing agent or prior docetaxel.

5. Intervention & Treatment Plan

5.1 Investigational Regimen * Dosage/Frequency: PF-08046054 administered via IV infusion (injected directly into the veins) twice during each 21-day cycle OR Docetaxel administered via IV infusion once during each 21-day cycle. *** Route of Administration:** Intravenous (IV). *** Duration of Treatment:** Study participation may be up to 5 years if the participant's NSCLC is responding to treatment.

5.2 Concomitant & Prohibited Therapy * Permitted: Standard supportive care medications (e.g., antiemetics) as per institutional guidelines. *** Prohibited:** Concurrent investigational agents, additional systemic anti-cancer therapies, or live vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor biopsy/archival tissue for biomarker (PD-L1/AGA) analysis, Baseline Scans
Baseline	Day 0	Randomization, First Dose
Interim	Every 21 Days	Treatment administration (Days 1 and/or 8 of 21-day cycles depending on arm), Safety Labs, Efficacy Assessment (Imaging), QoL Questionnaires
End of Study	Up to 5 Years	Final Efficacy Scans, Exit Interview, IP Accountability, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. **Overall Survival (OS):** Defined as the time from the date of randomization to the date of death due to any cause. 2. **Progression-Free Survival (PFS) assessed by BICR:** Defined as the time from the date of randomization to the date of

the first documentation of objective progressive disease (PD) assessed by Blinded Independent Central Review (BICR) per RECIST v1.1, or death due to any cause, whichever occurs first.

7.2 Secondary Outcomes 1. Objective Response Rate (ORR) as assessed by BICR. 2. Progression-Free Survival (PFS) as assessed by Investigator. 3. Objective Response Rate (ORR) as assessed by Investigator. 4. Change in Quality of Life (QoL), functioning, and lung cancer symptom response.

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Continuous monitoring through clinic visits every 21-day cycle, graded using NCI CTCAE v5.0 criteria.
 - **Serious Adverse Events (SAEs):** Reported to the sponsor and regulatory authorities within 24 hours of investigator awareness.
 - **Data Monitoring Committee (DMC):** An independent DMC will review unblinded safety and efficacy data periodically to ensure ongoing subject safety and trial validity.
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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy (OS, PFS); Safety Set for all participants who receive at least one dose of study medication.
 - **Sample Size Justification:** Powered to detect a statistically significant improvement in OS and PFS between the PF-08046054 and docetaxel arms ($\alpha = 0.05$, sufficient power for targeted enrollment of $N=680$).
 - **Handling of Missing Data:** Censoring rules for time-to-event endpoints (OS, PFS) will be applied per standard FDA/EMA oncology guidelines.
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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
 - **Monitoring Plan:** Routine onsite and remote monitoring visits by clinical research associates (CRAs) to verify protocol adherence, source data, and drug accountability.
 - **Audit Trail:** Maintained via validated 21 CFR Part 11 compliant Electronic Data Capture (EDC) systems, with central BICR for imaging data.
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11. Study Identifiers & Governance

- **Primary ID:** {{C5851005}}
 - **Registry Number:** {{NCT07144280}}
 - **Posting ID:** {{786873580939270071}}
 - **Trial Status:** {{RECRUITING - Currently recruiting participants}}
 - **Sponsor/Lead Organization:** {{Pfizer}}
 - **Study Title:** {{PADL1NK-005}}
 - **Official Regulatory Title:** {{A RANDOMIZED, PHASE 3, OPEN-LABEL STUDY TO EVALUATE PF-08046054/SGN-PDL1V VERSUS DOCETAXEL IN ADULT PARTICIPANTS WITH PREVIOUSLY-TREATED PROGRAMMED CELL DEATH LIGAND 1 (PD-L1) POSITIVE NON-SMALL-CELL LUNG CANCER (NSCLC)}}
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12. Clinical Framework (NSCLC Specific)

- **Disease Areas:** {{Non-small Cell Carcinoma, Non-Small Cell Lung Cancer Metastatic, Non-Small Cell Lung Carcinoma}}
 - **Preferred UMLS Name:** {{Advanced Non-Small Cell Lung Cancer}}
 - **Semantic Type & Condition:** {{Neoplastic Process / Carcinoma}}
 - **Diagnosis Threshold:** {{Unresectable Stage IIIB/IIIC or Metastatic Stage IV (M1a, M1b, M1c); PD-L1 expression $\geq 1\%$ }}
 - **Medical Methodology:** {{Targeted Antibody-Drug Conjugate (ADC) vs. Standard Chemotherapy (Docetaxel: Docefrez, Taxotere)}}
 - **Optimization Target:** {{Overall Survival (OS) and Progression-Free Survival (PFS) by BICR}}
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13. Study Procedures & Methodology

- **Management Pathway:** {{Randomized 21-day cycle infusions (PF-08046054 IV twice/cycle vs. Docetaxel IV once/cycle)}}
 - **Education Protocol (HCP):** {{Investigator training on ADC-specific toxicities and response evaluation criteria (RECIST v1.1)}}
 - **Education Protocol (Patient):** {{Informed consent, infusion schedule orientation, and symptom reporting guidelines}}
 - **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) for efficacy endpoints and independent DMC for safety}}
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14. Observation & Follow-Up Schedule

- **Total Duration:** {{Up to 5 years}}
- **Onsite Visit Cadence:** {{Every 21 days (each treatment cycle)}}
- **Remote Monitoring Frequency:** {{As specified by the clinical monitoring plan}}

- **Checklist Review Interval:** {{Prior to each infusion cycle}}